Weekly cetuximab + FOLFIRI

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · chemotherapy and antibody

Active medicines: Cetuximab + Irinotecan + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Antihistamine	As prescribed in the local cetuximab premedication plan	At least one hour before cetuximab Repeat for subsequent cetuximab infusions; assess prior infusion reactions.
Dexamethasone	8 mg PO once, subject to reconciliation with the cetuximab premedication prescription	At least one hour before cetuximab on Day 1 This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose.
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.
Atropine sulphate	250 micrograms subcutaneously	Before irinotecan when prescribed; assess acute cholinergic symptoms Avoid in glaucoma. Use with caution in asthma; review suitability before prescribing. This treats/prevents the acute cholinergic syndrome; it is not the delayed-diarrhoea rescue medicine.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Provide a written irinotecan delayed-diarrhoea rescue and escalation plan before treatment; distinguish it from immediate infusion-related cramps/sweating. Fever, severe pain, dehydration or diarrhoea not responding to the prescribed plan needs urgent review.
- Confirm line care, pump contact details and disconnection arrangements; report chest pain, severe early fluorouracil toxicity or pump/line problems urgently.
- NCCP p1 permits cessation of observation after two tolerated cetuximab doses, whereas Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Reconcile and document the local observation plan; do not promise early discharge.
- Folinic acid follows the bolus 5-FU hold/omission. Reconcile dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) inhibitors, DPD status and suspected fluoropyrimidine cardiotoxicity before administration.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00328 v7 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-07 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required